

Care Coordination Evaluation - Record Review

MCG Health
Transitions of Care
28th Edition

TC: TC-1025 (TC)

Print View

- [Assessment](#)
- [Plan of Care](#)
- [Evidence Summary](#)
- [References](#)
- [Footnotes](#)

Assessment

[Return to top of Care Coordination Evaluation - Record Review - TC](#)

Q1. Case Manager: Based on the patient's age, sex, and condition, are there any recommended tests needed? (Check all that apply.)

- Blood pressure [P1]
- Blood sugar [P1] [P2]
- Body mass index (BMI) [P1]
- Cholesterol [P1] [P2]
- Colorectal [P1]
- Dilated retinal examination [P1]
- HbA1c [P1] [P2]
- Laboratory work, specify: _____ [P1] [P2]
- Mammogram [P1]
- Pap test [P1]
- Vaccinations, specify: _____ [P1] [Vaccinations, Adult](#)  TC
- X-rays or imaging tests, specify: _____ [P1]
- Other, specify: _____ [P1]
- None
- Unknown [P1]

Q2. Case Manager: Has it been more than 12 months since the patient's last primary care visit?

- Yes [P1]
- No
- Unknown [P1]

Q3. Case Manager: Does the patient need refills of any medications?

- Yes [P3]
- No
- Unknown [P1]

Q4. Case Manager: Are there any considerations that need to be made when meeting with the patient? (Check all that apply.)

- Abuse known or suspected (emotional or physical) [P1]
- Behavioral health problems, specify: _____ [B1] [P1] [Behavioral Health and Cognition Evaluation with Caregiver](#)  TC
- Caregiver support limited [B2] [P1] [P4]
- Cognitive problems, specify: _____ [B3] [P1] [Behavioral Health and Cognition Evaluation with Caregiver](#)  TC
- Cultural considerations, specify: _____ [B4] [P1] [Cultural Practices](#)  TC
- Decision support for treatment, surgery, or procedure, specify: _____ [P1] [P5]
- Financial problems, specify: _____ [B5] [P1] [P6] [Financial Status and Benefits](#)  TC

Transitions of Care >Care Coordination Evaluation - Record Review (TC-1025)

- Hearing problems [P1] [Hearing](#)
- Home care involvement [P1]
- Housing or homelessness, specify: _____ [B8] [P1] [P7]
- Intellectual disability, specify: _____ [B3] [P1]
- Language translation needed [B9] [P1] [Language](#) TC
- Legal issues [P1]
- Nonadherence history [P1] [P8]
- Physical disability, specify: _____ [P1] [Physical Impairments](#) TC
- Specialty provider involvement [P1]
- Substance misuse [P1] [Alcohol Use](#) TC [Substance Use](#) TC
- Tobacco use [P1] [Tobacco Use](#) TC
- Vision problems [B10] [P1] [Vision](#) TC
- Other, specify: _____ [P1]
- None
- Unknown [P1]

Plan of Care

[Return to top of Care Coordination Evaluation - Record Review - TC](#)

Barriers

[Return to top of Care Coordination Evaluation - Record Review - TC](#)

- B1. Mental illness
- B2. Caregiver inadequate
- B3. Intellectual disability
- B4. Cultural barrier
- B5. Financial barrier
- B6. Health literacy
- B7. Hearing impairment
- B8. Housing inadequate
- B9. Linguistic barrier
- B10. Vision impairment

Problems

[Return to top of Care Coordination Evaluation - Record Review - TC](#)

- P1. Potential gap in patient care
 - **Goal:** Potential gaps in care evaluated and addressed(1)(2)
 - **Interventions:**
 - **RECONCILE:** Recent follow-up care orders and results from all care providers
 - **RECONCILE:** Care plan from all care team members, patient's ability and preferences, and available resources to support plan
 - **ASSIST:** Identifying patient's goal priorities, willingness/ability to achieve goals, and preferences for communication and follow-up
 - **ASSIST:** Goal setting that is specific, measurable, achievable, relevant, and with target completion time (eg, short/long-term, prioritized)
 - **ASSIST:** Identifying and resolving barriers
 - **ASSIST:** Identifying gaps in care and communicating with primary care provider prior to appointment:
 - Identifying recent missed appointments
 - Identifying medications needing refilled
 - Identifying needed testing or preventive services
 - **ASSIST:** Appointment scheduling with provider for evaluation and orders for clinical services, if indicated
 - **ASSIST:** Support system (eg, family, friends) identification, enlistment, and education
 - **EDUCATE:** Potential gaps in care, barriers, and ways to manage or resolve them
 - **COORDINATE:** Communication between care team members and care plan reconciliation
 - **COORDINATE:** Outpatient imaging, tests, or procedures, if indicated
 - **COORDINATE:** Home care referral, if indicated(3)
 - **COORDINATE:** Community services referral, as needed
 - **COORDINATE:** Protective services or legal authorities, as needed

Transitions of Care >Care Coordination Evaluation - Record Review (TC-1025)

- **COORDINATE:** Follow-up to evaluate patient and referral status, identify problems, and evaluate need for further assistance(4)(5)
- **SEND:** Written information on where and when to receive preventive health services
- **Goal:** Preventive services completed(1)(6)
 - **Interventions:**
 - **RECONCILE:** Current clinical care (eg, medication changes, laboratory tests)
 - **ASSIST:** Identifying patient's goal priorities, willingness/ability to achieve goals, and preferences for communication and follow-up
 - **ASSIST:** Goal setting that is specific, measurable, achievable, relevant, and with target completion time (eg, short/long-term, prioritized)
 - **ASSIST:** Identifying and scheduling appointment with primary care provider
 - **ASSIST:** Appointment scheduling with provider for evaluation and orders for preventive services, if indicated
 - **RECONCILE:** Recent test results with primary care provider records, if needed
 - **EDUCATE:** Importance of preventive services to check health status
 - **EDUCATE:** How and where to receive preventive services
 - **EDUCATE:** Which provider will follow results of preventive services
 - **COORDINATE:** Protective services or legal authorities, as needed
 - **COORDINATE:** Community services referral, as needed
 - **COORDINATE:** Follow-up contact to evaluate patient status with getting preventive services completed and identify need for further assistance(4)(5)
 - **SEND:** Written information on where and when to receive preventive health services
 - **SEND:** [How to prepare for your colonoscopy](#)  (Spanish)
 - **SEND:** [How to prepare for your mammogram](#)  (Spanish)

P2. Laboratory work coordination needed

- **Goal:** Laboratory work completed(2)(1) 
- **Interventions:**
 - **RECONCILE:** Laboratory work needed from all providers
 - **COORDINATE:** Laboratory testing, if indicated
 - **ASSIST:** Identifying laboratory location and scheduling appointment, if needed
 - **ASSIST:** Identifying transportation resources and accessing as needed
 - **EDUCATE:** Purpose and importance of getting laboratory work
 - **EDUCATE:** Follow-up with provider for laboratory results
 - **COORDINATE:** Follow-up contact with provider after laboratory work done to evaluate patient's status
 - **COORDINATE:** Communicate laboratory results to all relevant providers.
 - **COORDINATE:** Follow-up to review laboratory results and follow-up care needs, and to evaluate referral status or need for further assistance(5)(7)
 - **SEND:** Reminder on laboratory work due date, location, and provider follow-up

P3. Medication information needed

- **Goal:** Medication need, use, and management evaluated and problems resolved(2)(8)(1) 
- **Interventions:**
 - **RECONCILE:** Medication list. See [Medication Reconciliation Tool](#)  SR for more information. 
 - **ASSIST:** Creation of current and reconciled medication list
 - **ASSIST:** Providing reconciled medication list with all providers. See [Medication Reconciliation Tool](#)  SR for more information.
 - **ASSIST:** Identifying resources to pay for medications, if needed
 - **ASSIST:** Identifying activities that promote medication adherence
 - **ASSIST:** Communication with treating provider when another provider (eg, emergency or specialty provider) prescribes new medication
 - **ASSIST:** Appointment scheduling with provider to evaluate treatment outcomes
 - **EDUCATE:** **Reason for taking medication(9):**
 - Expected effects if the medication is working, and expected time for effects to occur
 - How long medication will likely need to be taken
 - Risks of nonadherence
 - **EDUCATE:** Information on all medications(8)(9)(10)(11):
 - Medication purpose, dose, effects/side effects, and considerations (eg, managing missed doses, interactions with food/other drugs)
 - Name of provider who ordered medication
 - Administration techniques

Transitions of Care >Care Coordination Evaluation - Record Review (TC-1025)

- skin color, hemorrhage, seizures, swelling of face, lips, tongue, or throat)
 - Common side effects include gastrointestinal changes, rash, dizziness, and headache.
 - Do not stop long-term medications abruptly due to risk of withdrawal symptoms.
 - Medication safety (eg, proper storage and disposal)(12)
 - Monitoring (eg, vital signs, laboratory checks) needed while taking medication
 - Talk to provider before taking any over-the-counter medications, vitamins, minerals, or herbal or dietary supplements.
 - Carry a list of medications at all times, particularly when visiting healthcare providers.(11)
 - **EDUCATE:** Ways to improve following medication routine (eg, pillboxes, alarm reminders)
 - **EDUCATE:** Pharmacy assistance programs, such as:
 - Centers for Medicare and Medicaid Services (CMS): www.medicare.gov/drug-coverage-part-d
 - Medicine Assistance Tool: <https://medicineassistancetool.org>
 - NeedyMeds: www.needymeds.org
 - Individual company websites
 - **COORDINATE:** Follow-up on the medication plan:
 - Contact with prescribing providers to verify medication orders
 - Contact with pharmacy to confirm patient receipt of medication
 - Contact with patient to evaluate adherence to medication plan and need for more assistance
 - Laboratory testing, if indicated(13)
 - **COORDINATE:** Home care referral, if indicated
 - **COORDINATE:** Follow-up to evaluate adherence or problems with medication, provide additional education and support, and identify need for further assistance(5)(7)
 - **RECONCILE:** Communicate reconciled medication list to all providers.
 - **SEND:** [Managing your medicines \(Illustrated\)](#)  (Spanish)
 - **Goal:** Medication safety understood and medication safety implemented(8)
 - **Interventions:**
 - **RECONCILE:** Medication list. See [Medication Reconciliation Tool](#)  SR for more information.
 - **ASSIST:** Communication with treating provider when another provider (eg, emergency or specialty provider) prescribes new medication
 - **ASSIST:** Communication with primary provider about prescription issues:
 - Polypharmacy
 - Food-drug and drug-drug issues
 - Adherence problems
 - **EDUCATE:** Medication safety instructions(14)(15):
 - Discard outdated medication.
 - Do not share prescription medication.
 - Store out of reach of children and in childproof containers.
 - Fill all prescriptions at the same pharmacy.
 - Store properly (in original containers and with other needs such as refrigeration or protection from sunlight).
 - Check with provider before taking over-the-counter medications, vitamins, minerals, or herbal or dietary supplements.
 - How to dispose of unused or expired medication
 - **EDUCATE:** Administration techniques
 - **EDUCATE:** De-prescribing or cessation of medications safely (eg, pharmacy-supported, provider monitoring planned)(16)
 - **COORDINATE:** Home care referral, if indicated
 - **COORDINATE:** Follow-up to evaluate adherence or problems with medication, provide additional education and support, evaluate referral status, and identify need for further assistance(5)(7)
 - **RECONCILE:** Communicate reconciled medication list to all providers.
 - **SEND:** [Managing your medicines \(Illustrated\)](#)  (Spanish)
- P4. Potential complication: caregiver needed to support patient's self-care**
- **Goal:** Caregiver support available and provided for safe transition to next level of care(17)(18)
 - **Interventions:**
 - **ASSIST:** Support system (eg, family, friends) identification, enlistment, and education
 - **ASSIST:** Deploying strategies to reduce caregiver burden, as needed (eg, healthcare benefits, support groups, family leave)(19)
 - **EDUCATE:** Patient's self-care needs and goals for care at next level of care
 - **EDUCATE:** When and how to get help for problems with patient or patient care
 - **EDUCATE:** Caregiver's anticipated role (eg, assisting with ADL, performing treatments, managing appointments)

Transitions of Care >Care Coordination Evaluation - Record Review (TC-1025)

- Identification of other caregivers
- Consideration of adult day care or respite program
- Self-care needs (eg, medical, behavior health, social support)
- **COORDINATE:** Care team meeting, if needed
- **COORDINATE:** Educational program referral for caregiver
- **COORDINATE:** Social worker referral, as needed
- **COORDINATE:** Community services referral, as needed
- **COORDINATE:** Follow-up contact to review caregiver resource needs, referral status, and need for additional support
- **SEND:** [Caregiver tips](#)  (Spanish)

P5. Potential complication: patient unable to make decision for care

- **Goal:** Patient will be informed of options, goals, benefits, risks, and costs associated with care decision.(22)(23) 
- **Interventions:**
 - **RECONCILE:** Patient's preferences and goals with care options, including:
 - Patient's ability and capacity for care with patient's preferences
 - Communicate care plan changes to entire care team.
 - Available and able caregiver at next level of care
 - Available caregiver support and that person's preferences
 - **ASSIST:** Identifying care options available to patient to safely meet care needs
 - **ASSIST:** Identifying goals and desired outcomes of care
 - **ASSIST:** Identifying and resolving barriers
 - **ASSIST:** Identifying contradictions between what patient wants and what they are doing or planning
 - **EDUCATE:** Care options available to safely meet care needs
 - **EDUCATE:** Benefits, risks, and costs of care options being explored
 - **COORDINATE:** Care team meeting, if needed(24)(25)
 - **COORDINATE:** Social worker referral, as needed(26)
 - **COORDINATE:** Community services referral, as needed
 - **COORDINATE:** Follow-up contact to evaluate status with decision making, potential problems secondary to indecision, referral status, and need for further assistance(5)(7)
- **Goal:** Patient (or designee) will make care decision.(2)(22)
 - **Interventions:**
 - **ASSIST:** Identifying appropriate support person (designee) to make patient care decision if patient unable to[A]
 - **ASSIST:** Completing legal documentation (eg, medical power of attorney), if needed
 - **ASSIST:** Identifying and resolving barriers
 - **EDUCATE:** Benefits, risks, and costs of care option selected
 - **COORDINATE:** Social worker referral, as needed(26)
 - **COORDINATE:** Legal aid
 - **COORDINATE:** Communication of care decision to care team members
 - **RECONCILE:** Care plan from all care team members, patient's ability and preferences, and available resources to support plan
 - **COORDINATE:** Follow-up contact to evaluate status of care decision, referral status, and need for further assistance(5)(7)

P6. Potential complication: financial issues present

- **Goal:** Financial resources identified and sufficient to continue care(27)(28) 
- **Interventions:**
 - **ASSIST:** Identifying healthcare benefits coverage from payers
 - **ASSIST:** Identifying availability of and eligibility for alternative financial resources
 - **ASSIST:** Communicating information on patient's healthcare benefits coverage with treatment team
 - **ASSIST:** Appointment scheduling and completing paperwork to support applications for financial support
 - **EDUCATE:** Healthcare benefits and out-of-pocket expenses
 - **EDUCATE:** Resources for financial assistance:
 - Administration for Children & Families: www.acf.hhs.gov
 - Centers for Medicare and Medicaid Services (CMS): www.medicare.gov/drug-coverage-part-d
 - Medicine Assistance Tool: <https://medicineassistancetool.org>
 - NeedyMeds. Find help with the cost of medicine: www.needymeds.org
 - Individual pharmaceutical company websites
 - **COORDINATE:** Financial assistance referral; examples include:
 - Area Agency on Aging
 - Disease advocacy

Transitions of Care >Care Coordination Evaluation - Record Review (TC-1025)

- Government agencies
- Legal aid
- Long-term disability application
- Medicaid application
- Medicare application
- Pharmacy assistance programs
- Short-term disability application
- Supplemental Security Income (SSI) or disability benefits application
- Veterans Affairs (VA) benefits application
- **COORDINATE:** Social worker referral, as needed(26)
- **COORDINATE:** Follow-up for evaluation of referral status and need for further assistance(4)(5)
- **Goal:** Financial issues addressed(27)
 - **Interventions:**
 - **RECONCILE:** Benefits from multiple sources, if needed
 - **ASSIST:** Obtaining preauthorization from health plan provider, if appropriate
 - **EDUCATE:** Source and type of financial support, how to access, and if follow-up needed
 - **COORDINATE:** Follow-up for evaluation of use of support programs or ongoing need for further assistance(4)(5)

P7. Potential complication: housing insufficient for patient care needs

- **Goal:** Housing issues addressed(27)(29)(N)
 - **Interventions:**
 - **ASSIST:** Identifying emergency housing, if needed
 - **ASSIST:** Identifying housing programs
 - **ASSIST:** Applying for available housing programs
 - **EDUCATE:** Available programs, including costs, wait times, and contacts
 - **COORDINATE:** Housing program referral
 - **COORDINATE:** Social worker referral, as needed(26)
 - **COORDINATE:** Follow-up contact to evaluate housing status, referral status, and need for further assistance(4)(5)

P8. Unhealthy behaviors potentially causing complications with treatment plan

- **Goal:** Information, support, and reinforcement of healthy behaviors to facilitate treatment plan understood(23)
 - **Interventions:**
 - **RECONCILE:** Treatment plan with patient's willingness and ability to facilitate in presence of unhealthy behaviors
 - **ASSIST:** Identifying types of unhealthy behaviors patients may exhibit, such as(30):
 - Aggressiveness, hostility, or abusiveness
 - Intentional nonadherence
 - Ineffective coping (eg, denial, poor insight, unmanageable anger, low self-esteem)
 - Self-care neglect (eg, not bathing, dressing, eating)
 - Social isolation or withdrawal
 - **COORDINATE:** Communication to care team members regarding potential problems with treatment plan related to patient's unhealthy behaviors
 - **ASSIST:** Appointment scheduling with care providers if unhealthy behaviors causing problems with patient's treatment plan
 - **ASSIST:** Creating behavior support plan to help facilitate treatment, including:
 - Identifying patient's goal priorities, willingness/ability to achieve goals, and preferences for communication and follow-up
 - Identifying factors that contribute to unhealthy behaviors and alternative behaviors that may help patient manage
 - Identifying support people who can help patient with behavior changes
 - Setting limits on patient's behavior and withdrawing attention from patient if behavior presents
 - Creation of emergency action plan if patient's unhealthy behavior escalates (eg, patient risk for harm to self or others)(31)
 - Providing positive reinforcement of healthy behaviors
 - **EDUCATE:** Unhealthy behaviors and potential consequences for treatment or outcomes
 - **EDUCATE:** Goals to help overcome unhealthy behaviors, including interventions, steps, and time frames
 - **EDUCATE:** Alternative coping mechanisms to help patient with behavior changes, such as(32)(33):
 - Social participation (eg, community groups, social organizations, family gatherings)
 - Recreation or hobbies participation
 - Behavioral health support (eg, counseling), role and benefit
 - Relaxation techniques (eg, deep breathing, guided imagery, music)
 - Stress management (eg, relaxation and coping techniques)
 - **EDUCATE:** When and how to get help for problems(34)

Transitions of Care >Care Coordination Evaluation - Record Review (TC-1025)

- Be aware of caregiver's personal feelings and their impact on patient's behavior.
- Support the patient's self-responsibility (eg, independence with hygiene, taking medications, decision making).
- Know signs of escalating behavior problems (eg, risk for harm to self or others) and when and how to get help.
- Use agitation reduction techniques (eg, avoid overstimulation, provide privacy, encourage activities, learn to de-escalate).(35)
- De-escalation techniques (eg, caregiver remains calm and provides emotional support, do not argue or threaten, stay serious, announce actions beforehand)(30)(36)
- **COORDINATE:** Behavioral health services referral, if indicated
- **COORDINATE:** Psychiatrist referral, if indicated
- **COORDINATE:** Recreational therapy referral, if indicated
- **COORDINATE:** Protective services or legal authorities, as needed
- **COORDINATE:** Follow-up to evaluate status with behavior changes, treatment plan, referrals, and need for further assistance(5)(7)
- **SEND:** Tips for good mental health  (Spanish)
- **SEND:** Caregiver tips  (Spanish)

Evidence Summary

[Return to top of Care Coordination Evaluation - Record Review - TC](#)

Quality standards: Standards indicate that programs should have a process for identifying barriers to patients meeting goals or adhering to the case management plan and for following up with patients on the status of referrals. Programs should ensure that during care transitions the care plan is re-evaluated, modified, and communicated to appropriate care team members (eg, providers, caregivers) as needed to support the care transition.(37)

Quality standards: Standards indicate that programs should have a process for identifying barriers to patients meeting goals or adhering to the case management plan and for following up with patients on the status of referrals. During transitions, medications should be documented with the patient and designee, and discrepancies should be communicated to the appropriate provider.(37)

Outcomes: Several studies support the role of the pharmacist in medication reconciliation to identify and resolve pharmacy-related problems and improve outcomes (eg, prevention of medication-related adverse events, discontinuation of inappropriate medication).(38)

Quality standards: Standards indicate that caregiver resources should be evaluated. Programs should ensure that during care transitions the care plan is re-evaluated, modified, and communicated to appropriate care team members (eg, providers, caregivers) as needed to support the care transition.(37)

Quality standards: Standards indicate that patients and their designees should be informed about the care transition process, the transition should be done in collaboration with the discharge team, and the case management plan should be reassessed and modified as needed. Programs should have a process for identifying barriers to patients meeting goals or adhering to the case management plan and for following up with patients on the status of referrals. Programs should ensure that during care transitions the care plan is re-evaluated, modified, and communicated to appropriate care team members (eg, providers, caregivers) as needed to support the care transition.(37)

Quality standards: Standards indicate that programs should have a process for identifying barriers to patients meeting goals or adhering to the case management plan and for following up with patients on the status of referrals.(37)

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[Return to top of Care Coordination Evaluation - Record Review - TC](#)

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Transitions of Care >Care Coordination Evaluation - Record Review (TC-1025)

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[Transitions of Care](#) > Care Coordination Evaluation - Record Review (TC-1025)[view abstract...](#)

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Footnotes

[Return to top of Care Coordination Evaluation - Record Review - TC](#)

[A] Patients who are confused, have cognitive disabilities, or exhibit psychotic behavior may not be appropriate to make care decisions but should still be informed about plans and included as much as possible in the planning process.(39) [A in Context Link [1](#)]

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